



WC/2024/227 Cory Shepherd v WCR
Disclosure from witness conferencing - Tammy Reese

WCRS

Workers' Compensation Regulatory Services
worksafe.qld.gov.au



From: [David Hall](#)
To: [Renee Matheson](#)
Subject: CM: FW: WC/2024/227 - Cory Shepherd v Workers' Compensation Regulator - Request for documentation
Date: Tuesday, 8 July 2025 10:53:05 AM
Attachments: [image002.png](#)
[image003.png](#)
[image004.png](#)
[Fwd Communication Book Update.msg](#)
[Communication Book 20.05.2024.msg](#)
[RE Catch up 130pm - 15.11.23.msg](#)
[RE Increase of hours and Workplace issues.msg](#)
[RE Increase of hours and Workplace issues.msg](#)
[Re Increase of hours and Workplace issues.msg](#)
[Re Increase of hours and Workplace issues.msg](#)
[RE Increase of hours and Workplace issues.msg](#)
[RE Increase of hours and Workplace issues.msg](#)
[Rostered shifts Cory S. past 8 months.xlsx.msg](#)
[Re Roster Concerns.msg](#)
[RE Roster Concerns.msg](#)
[Roster Concerns.msg](#)
[FW Roster Concerns.msg](#)
[Re Increase of hours and Workplace issues.msg](#)
[RE Office Hours and Departmental Directives.msg](#)
[Re Office Hours and Departmental Directives.msg](#)
[RE Office Hours and Departmental Directives.msg](#)
[Switchboard Manager - On call and Hours..msg](#)

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David Hall

A/Director | Employee Health Safety & Wellbeing
Metro South Human Resources

Metro South HR

m. 0412058080

e. David.hall@health.qld.gov.au | metrosouth.health.qld.gov.au

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>
Sent: Monday, July 7, 2025 2:37 PM
To: David Hall <David.Hall@health.qld.gov.au>
Subject: RE: WC/2024/227 - Cory Shepherd v Workers' Compensation Regulator - Request for documentation

Good Afternoon David,

I have been requested below to send the following emails and information to Renee Matheson with regards to the Workers Compensation Appeal in relation to a staff member as part of this review. Thus our HR dept suggested when I checked to see if it is approved to send this information that I just check out with yourself if the contents of the email I have drafted to send to check if it is acceptable to send and share this information and requested emails. Happy for any feedback you may have. Some of my answer to their questions are in the email trail below in blue.

Kind Regards

Tammy

Hi Renee,

Please see the relevant information to your questions below in blue and requested information as well. If I have missed anything or if you have any further questions please let me know.

Kind Regards

Tammy

Tammy Reese

Director HIMS-LBH | Health Information Management Services

MSH Finance Services

Logan Hospital, MSH

Level 2, Building 1, Cnr Loganlea & Armstrong Rds, Qld, 4131

[Metro South Health](#) |   

e. tammy.reese@health.qld.gov.au | [Chat on Teams](#)

p. 07 3299 8934

a. Mon-Fri 8:30-5:00



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From: Renee Matheson <Renee.Matheson@oir.qld.gov.au>

Sent: Friday, July 4, 2025 11:36 AM

To: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Subject: WC/2024/227 - Cory Shepherd v Workers' Compensation Regulator - Request for documentation

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Good morning Tammy,

Thank you for taking the time to speak with Lisa and I yesterday in relation to this matter.

The information you have provided is very helpful and thank you for being willing to provide some additional documentation as mentioned in the conference yesterday.

From our notes from yesterday, could we please have the following information:

- Please confirm the start date of employment of Chloe Taylor as manager of the Switchboard team.
 - Chloe Taylor was acting in our role of Switchboard Manager on the following dates, noting she did eventually get the role permanently and remains in the role to date
 - > 4-4-22 to 17-5-22
 - > 28-7-22 to 16-9-22
 - > 31-1-23 to today
- Issue of communication book – Email from Chloe to Tammy in relation to 06/06/2023 event of comment being removed. (paragraph 8 SOFAC)
- Please see the first attachment above titled 'Fwd: Communication Book Update' which was sent to me from Chloe on the 6.6.23 at 4:06pm.
- Any additional emails Tammy may have in relation to communication book issue on or around February 2024. (paragraph 8 SOFAC)
 - The only other communication book email issue I can locate was in relation to I think to another staff member but this occurred around the 20.5.24 but I am not 100% sure if Chloe exactly knows whom was the author of the entry and I wonder if there may be a possibility it was not the employee Carolyn whom she original thought was the author of this entry.
- Details on meeting had with Mr Shepherd before July 2024 in relation to his start/finish times or absences (if available)
 - Please note I do have a summary of a meeting Chloe and I had with Cory in my office on the 15.11.23. I have attached a copy of the email appointment sent to Cory. Points that were discussed are as follows:
 - Giving feedback on the roster he submitted to the Business Case for Change process for Switchboard. Chloe thanks Cory for taking the time to submit a roster for consideration as part of the business case for change.
 - Discussion about Cory working on some sort of agreement with Switchboard staff that he was seeking their endorsement/feedback on. I think the background was some Switchboard staff had raised concerns with Chloe that Cory been asking staff to review his 12 hour draft roster and he was seeking their individual agreement to working a 12 hour roster. Chloe was hoping to get a better understanding from Cory about what was the agreement he was seeking from staff as some staff had reported they were a bit uncomfortable about being asked to agree with what was being proposed. Cory stated that it was just asking if staff agree to work 12 hour shifts and was just verbal and he took what staff say at face value and he was not forcing anyone to go either way and he would be happy to send it to Chloe but I think Chloe stated that was not necessary at the time. Chloe and I discussed that Cory just had to aware that different people had different circumstances and we just need to be respectful of these different circumstances and it may even be part-time staff and full-time staff may have different opinions on what would work best for themselves. It was discussed some staff may need time to consider what is being proposed and don't want to feel pressured to agree. Cory said he respected staff's decisions if they did not want to sign anything.

- I updated Cory about what the next steps were in relation to Business case for change and that we were currently collating the feedback from staff, reviewing roster feedback and reviewing the rosters presented and the next step would be hopefully a decision on what roster is recommended for approval or if another round of consultation is required, union consultation and then there would need to be an implementation document to discuss with staff what the roster would look like and dates of implementation and then the roster would be trialled and then reviewed after a certain period of time. I did ask some queries about his 12 hour roster and how he proposed that would work in terms of pay. I also advised that the main issue with the roster is there is not necessarily fairness to the part-timers for penalties when you compare the full-time penalties, but Cory stated he did try to balance this by statistical value. I also discussed that his roster did not include cross over shifts like we currently have and I explained why I thought that was important from a coverage point of view if one of the persons coming into shift could be potentially delayed for example due to traffic and I discussed why handover was important.
- Chloe discussed with Cory his start times being late and that of the shift and break times being sometimes too long and that she was also addressing this with other staff also. Chloe discussed that staff including Cory had been arriving late and also finishing early and that she had sent a recent email out about shift times and break times the previous week but it still seems to be a bit of an issue with some staff including Cory. Chloe said that she knew Cory had discussed one such occasion with her but just to remember the requirements about starting times, only taking allocated break times (and she discussed what these were, 10 mins first break, lunch 30 mins and 2 break 10 mins), working back if late to work but notifying Chloe of this. Chloe also discussed that Cory needed to remember to update the roster and following that process giving her a call. Cory stated he thought he did call Chloe but Chloe said he usually does but not always and that it was not just Cory she is reminding but other staff also.
- Cory asked Chloe to address the issue of staff using their phones as this was a concern and that he was tired of taking the calls when other are not. When queried about what are the other staff doing if they are not taking calls from the switchboard Cory said they may be answering work emails but that he was frustrated that they don't multitask like he does and answer calls whilst attending to emails, etc. Cory said some staff 'eight' their keyboard and this frustrated him, as he feels exhausted that sometimes others are not picking the same call volume. I advised if Cory feels this is an ongoing issue with certain an individual or individuals to please let Chloe know so that she can follow up on this with reviewing the relevant shift call stats and then following up with the staff member if a problem is found.
- Chloe raised concerns she had with Cory in relation to new processes in Switchboard and said something to the effect of that as much as she is willing to hear feedback and staff ideas that Cory just needs to be mindful that should Cory or other staff wish to introduce new processes or suggest different ways of doing things to just discuss it with Chloe first before making changes.
- Chloe discussed that if Cory had any other concerns at the present time which he said he did not but that should he ever have any to bring these to herself or

me at any time at all.

- Chloe was planning to arrange a discussion with Cory with myself there as support to Chloe upon his return after (after the mid June 24) as he was on sick leave at the time to have a discussion about a few behavioural issues from Cory, including giving adequate time to call in before a shift as the previous week I think he had given less than 2 hours notice (as per our Switchboard guidelines which is minimum 2 hours notice) before a shift start that he would not be attending his shift, not leaving the shift early, etc. but after the recent issue of Cory being on leave for a number of weeks and with Cory now not having any leave balance left SL or RL in lieu of sick leave it was decided a separate discussion item about Absenteeism that we were also planning on addressing was the first one Chloe needed to be addressed with Cory before discussing the other behavioural issues with Cory. Unfortunately, then Cory was off on leave so the two separate conversations did not happen occur at the time.
- Emails and exchanges relating to 07/08/2023 email Mr Shepherd sent to Tammy, Chloe, Tracey Smith and Patricia stating multi issues:
 - Tammy email to Chloe with 8 months of Mr Shepherd's roster to check.
 - Please see Cory's email that he sent to on the 7.8.23 at the bottom of the fourth attached email titled "RE: Increase of hours and Workplace issues" which is my response to Cory following that email.
 - Responses
 - Meeting details noted to have happened 29/08/2023
 - Please see the email I sent to Cory which summaries most items discussed at the meeting. Essentially from my notes our discussion included:
 - Rostering concerns that Cory had about his roster
 - Distribution of roster timeframe concerns
 - Concerns about Chloe
 - Request for additional shifts
 - Please note Cory did raise some roster concerns on the 1.5.24 and you will see his email attached, my email in response and also my reaching out to HR to get clarification on certain items Cory is referring to in his email
- 13/05/2024 issue – the pack – all emails and correspondence in relation to this issue (paragraphs 12 and 16 SOFAC)
 - Noted this email could be dated 15/05/2024
 - I have looked back at my notes regarding this time and would like to correct my statement I made 3.7.25 when we discussed this issue. Please note following receiving this email from Cory, Chloe rang me stating she was upset following receiving this email and she had contacted HR and spoke to Mackenzie Prichard. I then contacted HR to ask what should I do in regards to this request and rang HR but could not get Mackenzie but instead spoke to Brendon Punch from HR. Brendon assist me with suggesting wording to put in an email to Cory in reply recommending asking if he could retract the email.
 - Emails with HR Elise
 - Response from Mr Shepherd
- 15/5/24 email from Mr Shepherd to Tammy asking if Chloe's hours etc could be communicated to the entire team (paragraph 15 SOFAC)
 - emails regarding Tammy's advice to Mr Shepherd in relation to this and any response from Mr Shepherd

- emails that Tammy received from HR in relation to this (if any)
- on or about 15/5/24 the email from Chloe advising of her hours

If you have any questions or concerns in relation to the above, please do not hesitate to contact me.

Kind regards,

Renee Matheson
Senior Appeals Officer

Workers' Compensation Regulatory Services

Office of Industrial Relations

P 07 3842 2792 M 0460 021 030

150 Mary Street, Brisbane QLD 4000

PO Box 10119, Brisbane Adelaide St QLD 4000



From: Renee Matheson

Sent: Monday, 30 June 2025 12:06 PM

To: tammy.reese@health.qld.gov.au

Subject: WC/2024/227 - Cory Shepherd v Workers' Compensation Regulator - documents for conference

Good afternoon Tammy,

In readiness for our conference with you at 1pm on Thursday 3 July 2025, please see below allegations raised by Mr Shepherd that Lisa and I will be speaking with you about.

The conference is an informal chat, although confidential and not discussed with other witnesses, where Lisa and I can touch base and get some background in relation to events raised by Mr Shepherd.

Stressor 1 – Victimisation in the workplace

5. During his employment, Ms Taylor and Ms Reese would react negatively when Mr Shepherd raised workplace concerns.

6. On 4 April 2023, Mr Shepherd sent a text message to Ms Taylor complaining that because of the way he was rostered, he would earn \$1,500.00 less than his colleagues.

7. On 7 August 2023, Mr Shepherd sent an email to Ms Taylor and Ms Reese, raising concerns about his roster and his treatment in the workplace.

8. On or around February 2024:



Communications Book issues

WCRS

Workers' Compensation Regulatory Services
worksafe.qld.gov.au



From: [Chloe Taylor](#)
To: [Tammy Reese](#)
Subject: Fwd: Communication Book Update
Date: Tuesday, 6 June 2023 4:05:39 PM
Attachments: [image001.gif](#)
[image002.gif](#)
[image003.gif](#)
[image004.gif](#)
[image005.gif](#)
[image006.png](#)

Good afternoon Tammy,

Just letting you know that Cory Shepherd approached me in the switchboard room this afternoon after I sent an email regarding the communication book email (see below).

I was not aware of who put this entry in at the time but I took it out last week as it was clearly an indirect dig at the team and there is already a procedure to follow with this certain entry. I didn't agree it belonged in the communications book as we only use this book for important notices. On the page there was no instruction as to what to do with the information and no name signed on the sign off. Anyway, I was looking further into the book and read quite a few unnecessary comments from other staff about other notices - I don't agree with this and I just feel like it's being used as a "burn book" more than a professional tool that switchboard can use to communicate.

After Cory arrived for his afternoon shift, he read the email and questioned it in an aggressive tone and raised voice. I explained why I sent the email and he said he feels like he is being attacked. I didn't even know it was him that put this in the book. I asked him to explain why it was put in there and I gave him my input towards this. He wouldn't let me speak, talking over the top off me. I did raise my voice and asked him to please stop talking over the top of me. I put a stop to the discussion and told him if he would like to schedule a meeting with you and myself in regards to any issues we would be more than happy to discuss these with him.

I would also like to mention other staff have been coming to me and making me aware of comments he has been making towards management and rostering. Informing staff that Industrial Relations have been made aware of the rostering issues.

I just thought I would let you know,

Kind regards,
Chloe

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From: Chloe Taylor

Sent: Tuesday, June 6, 2023 9:57 am

To: Logan Switch

Cc: Peter Fitzpatrick ; Carolyn Jeffrey ; Cherrie Abbott ; Patricia Conaghan ; Kaymarie Harvey ; Cory Shepherd ; Stacey Sutton-Masson ; Sonya Willmann ; Taiha Zizek ; Cory Harrison-Jones ; Linda Hu ; Anisha Ismail ; Meegan Malsbury ; Jude Nelson

Subject: Communication Book Update

Good morning everyone,

Just a friendly reminder that the communication book is a tool for everyone to communicate important notices to the team. This is not used to simply put your point across or make any indirect comments towards the team. I was really disappointed reading through the communication book last week to come across a few unnecessary entries.

The sign off sheet has been updated. From now on, please write your name when you enter any new updates/information (there is a box for this on the new sheet) and please sign your name off once you have read the update/information at the start of your shift. If you are unclear about any of the entries please let me know or clarify with the staff member who put the important notice in book.

Any concerns or questions please let me know,

Kind regards,

Chloe

Chloe Taylor

A/Switchboard Manager | Switchboard Services

Logan Hospital | **Metro South Health**

Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



From: [Chloe Taylor](#)
To: [Carolyn Jeffrey](#)
Subject: Communication Book 20.05.2024
Date: Tuesday, 21 May 2024 10:46:45 AM
Attachments: [image001.gif](#)
[image002.gif](#)
[image003.gif](#)
[image004.gif](#)
[image005.gif](#)
[image006.png](#)
[processed-364C4A9C-7871-4808-956C-818D7CFBF5E5.jpeg](#)

Good morning Carolyn,
I was looking in the communication book and came across your entry that was put in yesterday 20.05.24.
Can I please ask what happened? The communication book is to be utilised for important notices regarding department/hospital updates and not to be used for such notices like attached. I have taken this out of the communication book as this is not the appropriate way to action your concern. There are other ways to raise concerns, for example please see me and we can discuss together.
Always here to support you Carolyn, please don't hesitate to speak to me about any issues you may have.
Kind regards,
Chloe

Chloe Taylor
Switchboard Manager | Switchboard Services
Logan Hospital | **Metro South Health**
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For your information

20/05/2024

Apparently, it is going around the gossip circle that I changed the numbers on the ED Cheat Sheet.

TO SET THE GOSSIP STRAIGHT. I DID NOT CHANGE ANY NUMBERS ON THE ED CHEAT SHEET I ADDED AN ED WARD CLERK NUMBER WHICH SOMEONE HAS REMOVED AND DID CHANGES IT WAS NOT ME. I GET NUMBERS TO HELP US WITHIN SWITCH.

Carolyn.



Increase of hours & Workplace issues

WCRS

Workers' Compensation Regulatory Services
worksafe.qld.gov.au



Renee Matheson

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>
Sent: Friday, 8 September 2023 11:42 AM
To: Cory Shepherd
Subject: RE: Increase of hours and Workplace issues

Hi Cory,

Thanks very much for your response and for your draft roster. We are happy to look at any rosters but we do ask that they fit the staffing lines we will have (e.g. we will shortly have 7 full-time lines, one 0.8 FTE, 3 x 0.7, 2 x 0.6 FTE and one 0.5 FTE), , must be fair and equitable to all staff ensuring fair distribution of penalties and be compliant with all industrial requirements. Thus if you do want to still present a roster could you please just review your roster and modify as required to suit the FTE we will have for Switch and you may just need to add some additional shifts to ensure there are enough for everyone and you will see from my released draft roster yesterday what sorts of shifts these might be e.g. an additional 12:00 to 8pm shift and an additional 0600-1400, etc. and then resend through to myself or HR. I am currently on leave until 3.10.23 (Marilla Fraser will be backfilling in my role) but upon my return I would be happy to catch up with you if you would like to talk further about your draft roster.

I am glad to hear things seem to be going well with Chloe and I am also glad to hear you applied for the additional shifts through the recent EOI.

Kind Regards
Tammy

From: Cory Shepherd
Sent: Monday, 4 September 2023 7:21 AM
To: Tammy Reese
Subject: Re: Increase of hours and Workplace issues

Good morning, Tammy,

I apologize for the delay in getting back to you regarding the rotational roster. I have attached a copy of the spreadsheet for you to view. I include many versions of the roster that you can change or edit as you please.

I appreciate your time and the information you have provided me regarding the grievance steps. I endeavour to resolve the issues I have raised, and I have also spoken to Chloe. We are seemingly on the path to working in a beneficial way and hopefully will not need to go through this process..

Moreover, the concerns I raised were more directed towards finding a solution for a positive workplace and a team environment. The time the roster is released is not the biggest concern. The issues are meeting the agreements and the purpose of the delay being discriminatory actions.

Additionally, I have also sent Chloe my official letter of response last week to increase my working hours prior to the cut-off date. I have attached it to this email.

I confirm that I am able and willing to work any roster that is presented to me, including the current schedule with full 24-hour availability. I have previously been employed offshore for four weeks at a time, working 12-hour shifts. Two weeks of those shifts were day shifts, and the other two weeks were night shifts. I have no issues with shift work.

Thank you,
Cory

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Date: Tuesday, 29 August 2023 at 6:57 pm

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Subject: RE: Increase of hours and Workplace issues

Hi Cory,

I am just following up with regards to our conversation on the 10.8.23. As discussed at this meeting you raised a number of roster concerns and I am now hoping you have had a chance to discuss these concerns with Chloe and please do discuss know you can discuss any concerns roster issues with Chloe as your line manager but please do not hesitate to raise any future roster concerns with myself you have tried that option or if you need any assistance to raise these concerns. Please note I encourage you to look at the roster when released in the first few days after it is released and get back to Chloe asap with any concerns you may have with regards to your rostering.

I noted you had concerns with the roster timeframe being released and upon review of previous 3 months of release dates for rosters and the majority were released in a timely manner with most well in advance of 14 days as per 9.13.1 but I do acknowledge your concern regarding that roster beginning 21.8.23 was sent out 1 day late and whilst we do aim ensure to comply with this there may be the rare occasions when circumstances may prevent us from doing so. However, please note we will try to ensure wherever possible that in future we do always comply with 9.13.1 and give Switchboard staff as much notice as possible of upcoming rosters.

In relation to your request for additional shifts where you confirmed in our discussion that you were potentially interested in increased perm full-time hours and you will see that the EOI has been opened up again for staff to apply if they do wish to request additional permanent FTE, just in case staff like yourself missed applying the first time, thus please do feel free to apply if you do want to request additional permanent hours.

With regards to the future roster and the draft you showed me as we discussed I would be happy to review your roster, so please feel free to send me a copy.

As discussed whilst I do recommend you do try and raise any concerns you do have with your line manager being Chloe and then myself, or if you have and your not happy with the outcome, you do have the option of lodging a Grievance directly with HR. As discussed at our meeting I have attached the [HR Policy E12 Individual Employee Grievances](#), which contains information regarding the grievance process.

You may submit your grievance to LBH_HR@health.qld.gov.au. Specifically, we ask that you provide the following details:

- Information relating to the instances that have aggrieved you. Who was involved (witness details?), what occurred, when and where did it occur?
- What outcome you are seeking from the grievance process?
- What attempts you have made to resolve the grievance locally?

As also discussed, as a Metro South Health employee you are able to access the Employee Assistance Program (EAP). Please see below contact information:

- Employee assistance offers a confidential counselling service which is free of charge to all employees of Metro South Health for up to six sessions per calendar year. Access to this service is by self-referral. Please contact LifeWorks on 1800 604 640 or Benestar on 1300 360 364.

Lifeworks website: [Employee Engagement & Wellbeing | Employee Assistance | LifeWorks AU](#)

Benestar website: [Contact Us | Benestar](#)

Please do not hesitate to let me know if you have any questions at all or if there is anything else I can assist you with.

Kind Regards
Tammy

Tammy Reese

Director HIMIS | Health Information Management Services, Digital Health & Informatics

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e. tammy.reese@health.qld.gov.au | metrosouth.health.qld.gov.au



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From: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Sent: Thursday, 10 August 2023 11:23 AM

To: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Subject: Re: Increase of hours and Workplace issues

Hey Tammy,

I am confirming that we will have a meeting today at 13:15pm.

The overall purpose of the meeting is to improve the workplace and prevent ongoing conflicts.

I will go over in a bit more details in the meeting .

Thanks for your time.

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Sent: Wednesday, August 9, 2023 9:54 am

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Subject: Re: Increase of hours and Workplace issues

Thanks Cory for the reply. I am out of the office today but I will send you an appointment for tomorrow's meeting this afternoon. I have advised Chloe that you would like another replacement shift for Tuesday's shift and once she has reviewed the roster she is going you get back to you with another shift.

Kind regards

Tammy

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From: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Sent: Tuesday, August 8, 2023 6:06:32 PM

To: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Subject: Re: Increase of hours and Workplace issues

Hi Tammy,

I can confirm that I will be in tomorrow 0700-1500 9th August. I was also looking to pick up another shift if possible.

I also confirm us meeting at 1:15pm on Thursday.

Thank you.

Kind regards,

Cory Shepherd

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From: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Sent: Tuesday, August 8, 2023 4:15:35 PM

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Subject: RE: Increase of hours and Workplace issues

Good Afternoon Cory,

Thank you for that further information. I am sorry that you have these concerns but I am sure we can work through the various concerns you do have and try to find solutions where possible. How about we have a meeting Thursday at 1:15pm in my office? I will send you an appointment if that timing works for you. The various issues I believe you would like to discuss are as follows (and please feel free to add or change to the list below):

- Rostering concerns for yourself
- Distribution of roster timeframe concerns
- Concerns about Chloe
- Request for additional shifts.
- Etc. Any other items you might like to discuss.

With regards to an additional shift to replace the shift you did not work Monday due to a rostering error I believe Chloe offered you the shift tomorrow Wednesday 9th 0700-1500, thus can I ask if you are interested in picking up that shift? I believe you will then have 8 shifts this fortnight if you were to pick up that shift, thus can I ask were you looking for another shift on top of those 8 to replace the shift missed today also?

Kind Regards

Tammy

From: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Sent: Monday, 7 August 2023 8:05 PM

To: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Subject: Re: Increase of hours and Workplace issues

Hi Tammy,

Every attempt with Chloe has been made. To make this oversight is not acceptable anymore, Chloe would not roster a night shift on a day shift and the expectation is the same with days off. I have already discussed with other employees and know that Chloe would say an oversight. I suppose rostering me for 6 weeks on the 0700 start which is unheard of an oversight also.

Additionally, The date Corey has given leave from switchboard does not prevent Chloe from distributing the upcoming roster, it's an unreasonable time delay to break Q-Health contractual agreement with its employees.

I have never made a formal complaint; this Does not mean this department is without error. I have worked with and for Queensland Health since 2014 Security, Nursing, Scanning and Switchboard.

Issues that I have been dealing with include Chloe tearing out communications or concerns from the communications book, work has not been done with diligence, pagers have been distributed without updating the numbers, titles, or the paging system, the second stage IMT list of pagers completely disregarded and security with one pager the rely on when tests have been sent for months.

I have attended, informed, or actioned and resolved all issues. For going above board without expectations Chloe has come into office and either screaming at me for telling my colleagues to check numbers that have not been completed and signed for two months or demanding an explanation as to why I have not included her in a simple email to Ellen to check up on a pager. I clearly follow instructions outlined only days prior with all other employees not including Chloe in email regarding updates why would this happen?

I understand that when there is a problem or issue with the direct line manager the formal process is to reach out to the director of the department. If the screaming, yelling, coercion of the truth and starting rumours is ok with you it's not with me. I have received at-least 4-5 messages from Chloe which I have only taken as an insult and a coercion of the truth.

The last and final is the rumour Chloe has been telling my colleagues is I didn't show up for my shift last week because I wasn't requested in a way I would like.

I requested the shift, and it was withheld from me to see if Stacey responded, I then was not advised, and others received overtime is the answer.

This can be confirmed by Kay, and I should not need to explain myself to everyone again following the previous rumour made up.

I need some rest I spent all yesterday in a proposal for the department, liaising with those who seek to improve it.

I am rostered tomorrow morning but for the second fortnight I won't be without a rest day if I am not compensated for it.

I request that tomorrow's shift be removed from my roster in accordance with the current policy following a night shift. I should have an alternative day of work provided to me.

I do apologise and I do not want to have a negative impact, previously is was in support of Chloe wishing her the best but enough is enough, the manager should not be at the centre of the problem and the rumour starting.

Everything can be confirmed by my work colleagues, and I did mention to Chloe that I want to have a meeting with you, but the meeting I would like is structured towards a solution to the problems outlined.

Thank you

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Sent: Monday, August 7, 2023 3:13 pm

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Subject: RE: Increase of hours and Workplace issues

Good Afternoon Cory,

Thank you for your email.

Can we catch up with you are next on shift, so we can discuss further the various concerns that you have raised. I believe you on shift Thursday day shift and I am in the office also, thus please do not hesitate to drop in for a discussion when you can. I am free from 9 – 10:30 and then 12:00-4:30 Thursday.

As Chloe is your current line manager and as such you are required to continue to communicate with Chloe for work related issues, shift concerns, leave, etc. but your welcome to CC me into any emails your are sending to Chloe or raise any particular concerns with myself following trying to address these first with your line manager and as discussed above I would be happy to be able to have a chance to have a discussion with you further about the concerns you have raised.

I do realise in this instance there was a rostering error that was accidentally made by Chloe with regards to night shifts and days off and Chloe will endeavour not to make this mistake again and whilst I this should not happen in the future can I ask that you do let Chloe know of any issues you identify with regards to your rostered shifts as soon as possible, to ensure corrections can be made straight away.

With regards to your request for additional increase of hours that is something we will have to get further clarification on from HR and I will have to get back to you about this.

Kind Regards

Tammy

Tammy Reese

Director HIMIS | Health Information Management Services, Digital Health & Informatics

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e. tammy.reese@health.qld.gov.au | metrosouth.health.qld.gov.au



Metro South Health acknowledges the traditional custodians of whose land and waters we provide health services and pays respect to Elders past, present and emerging.

From: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Sent: Monday, 7 August 2023 12:21 PM

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>; Tammy Reese <Tammy.Reese@health.qld.gov.au>; Patricia Conaghan <Patricia.Conaghan@health.qld.gov.au>; Tracey Smith <Tracey.Smith8@health.qld.gov.au>

Subject: Increase of hours and Workplace issues

Importance: High

Hi Chloe,

I have not agreed to be rostered like this. I have had enough of the continuous gossiping and the discrimination against me.

The union has already passed the agreement for rest days after night shifts, and I have not agreed to be rostered any differently.

You may not issue rest days only on penalty shifts or the weekend and for the second fortnight in a row, I will not be working a Monday morning with less than a 24-hour break following a night shift.

I will be in contact with Tammy directly regarding a lot of the current workplace issues.

Conflicting interests
Escalation and gossiping
Initiating workplace rumours
Adverse workplace behaviours
Workplace bullying and Harassment.

You are not in the office Monday to Friday and disregard all agreements in the workplace. Please give me some space and stop with any further communication as I have had enough of it.

Clear breach of the EB11 agreement.

9.13 Display of rosters

9.13.1 All employees (excluding employees where schedule 3 applies) shall be provided with a roster prescribing their pattern of work. **The roster shall be published at least 14 days in advance of each roster cycle.** The roster will be made available electronically, or where not possible, displayed in a convenient place accessible by all employees.

To withhold a roster to facilitate the needs of any staff member will be directly discriminating against all other employees. If an employee needs to take leave, then the available shifts on display are to be allocated to staff members to policy, this roster is not to be withheld for a distribution of your choosing.

10.6 Workplace Behaviour

10.6.1 The employer recognises that workplace bullying is a serious workplace issue which is not acceptable and must be eliminated.

10.6.2 All employees have the right to be treated fairly and with dignity in an environment free from adverse behaviours such as intimidation, humiliation, harassment, victimisation, discrimination, and bullying.

10.6.3 The employer recognises that adverse behaviours such as these are serious workplace issues, which are not acceptable and must be eliminated from the workplace.

10.6.4 The Code of Conduct for the Queensland Public Service applies to all employees covered by this Agreement. If it is substantiated that an employee is found to have been involved in the above adverse behaviours, this may be a breach of the Code of Conduct and they may be subject to a disciplinary process.

10.6.5 The employer supports the accepted industrial principle that all employees have the right to raise concerns with their employer about issues of bullying or workplace behaviour without fear of victimisation. Unions may refer instances of alleged victimisation directly to the EB11IG for attention.

10.6.6 The employer is committed to protecting and improving the health and wellbeing of all employees and their immediate family by providing employee assistance.

Furthermore, to meet the demands and the additional hours of the department both Patricia Conaghan and I would

like to formally adhoc an additional 2 shifts per fortnight. This formal request and information can be confirmed by Patricia Conaghan via email Patricia.Conaghan@health.qld.gov.au

11.7 Additional Permanent Hours for Part-time Employees

11.7.1 Part-time employees, following approval, may work more than their substantive (contracted) hours on an ad-hoc or temporary basis. Where an employee works more than their substantive (contracted) hours on a regular basis over a 12 month period, the employee may request an amendment to their substantive permanent part-time hours to reflect the increased hours worked. Such requests should not be unreasonably refused. 11.7.2 Any agreed permanent increase to an employee's substantive part-time hours is limited to a maximum of 64 hours per fortnight for part-time employees employed under the *Hospital and Health Service General Employees Award - State 2015*, or full-time.

5.2 Process to Address Absences within Operational Services

5.2.1 All absences (planned and unplanned) within Operational Services will be backfilled. The options to backfill may include but are not limited to:

(a) Offering additional ordinary hours to Part-Time employees,

- (b) Offering additional work to Casual employees,
- (c) The application of Relief Pool staff,
- (d) The use of Overtime,
- (e) Utilisation of temporary engagement (e.g. extended period of absence).

Thank you,

Cory Shepherd.

Cory Shepherd
Switch Board Operator
Metro South Queensland Health
Cory.shepherd@health.qld.gov.au

31 Aug 2023

Subject: Request to Increase Working Hours to Full Time Rotational Roster

Dear Chloe,

I am writing to express my interest in the increase of working hours and working Full Time hours within Switch Board Department in Qld Health, and to outline how my experience, abilities, knowledge, and personal qualities make me an ideal candidate for this role. Please find below a summary of my qualifications in relation to the key attributes and responsibilities, as well as the key skill requirements.

Experience and Abilities:

- I possess extensive experience in operating equipment and performing administrative duties. This background has equipped me with a solid foundation in effective communication and record-keeping. Additionally, I am highly skilled in various programs and databases, including the Cerner program, Alcatel Switchboard Console and Database, HBCIS Database, EDIS Emergency Department Web Database, Spok Messenger program, as well as numerous other systems and equipment. Furthermore, I possess exceptional problem-solving skills that allow me to respond effectively even in high-pressure situations utilizing alternative communication systems during emergency or security response situations.

Personal Qualities:

- Ethical decision-making, effective prioritization, and a responsible work ethic are some of the qualities that define my professional character. I am diligent, an effective listener capable of coordinating the operations of the Switch room, handling difficult calls, managing conflicts.

Teamwork:

- I strongly believe in the power of communication and collaboration. Forging a cohesive team environment is essential to me. Therefore, I actively share information with colleagues while offering proactive support and assistance whenever needed. I am committed to adhering to Queensland Health policies, procedures, standards, and relevant legislation.

Interdisciplinary Workplace:

- My approach towards interdisciplinary workplace settings is marked by understanding and respect for all stakeholders involved. Clear communication channels alongside setting expectations are vital components for effective conflict management.

Compassion:

- As a patient-focused professional providing service delivery with self-awareness and resilience. The needs of patients remain at the forefront throughout ambiguity or complexity that may arise.

High Performance:

- My action-oriented mindset and drive for results have consistently shown through constant review, understanding, and research. This commitment aligns with Metro North Health's vision and purpose of exceeding expectations for patients and stakeholders.

Integrity:

- I prioritize accountability and work diligently to ensure high ethical standards in decision-making and work output. Advocating the education for a positive and fair working environment is of utmost importance to me.

Considering my qualifications, I believe that I am well-equipped to undertake the responsibilities associated within the department. I have also expressed my willingness to take on additional night shifts as part of the roster and advocacy to become a union representative for the switchboard department. I look forward to contributing further to our team and services.

Yours sincerely,

Cory Shepherd

From: [Chloe Taylor](#)
To: [Cory Shepherd](#)
Cc: [Tammy Reese](#); [Tracey Smith](#)
Subject: RE: Increase of hours and Workplace issues
Date: Monday, 7 August 2023 1:43:04 PM
Attachments: [image001.gif](#)
[image002.gif](#)
[image003.gif](#)
[image004.gif](#)
[image005.gif](#)
[image006.png](#)

Hi Cory,

Thank you for your email, I acknowledge your concerns and I will be following up on your email. Tammy will be in contact to arrange a time to meet and discuss any issues you may like to raise.

My sincere apologies about your rostered Monday 7th 0700-1500 shift, I can confirm this was an oversight. I did release PP30 roster on Monday 19th July, may I please ask why you have not made myself or Tammy aware of this rostering error until now? Would you like me to roster you off tomorrow Tuesday 8th 0700-1500 to give you the required rest period and offer you Wednesday 9th 0700-1500 shift? Please let me know.

Kind regards,

Chloe

Chloe Taylor

A/Switchboard Manager | Switchboard Services

Logan Hospital | **Metro South Health**

Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



Renee Matheson

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>
Sent: Monday, 7 August 2023 5:11 PM
To: Chloe Taylor
Subject: Rostered shifts Cory S. past 8 months.xlsx
Attachments: Rostered shifts Cory S. past 8 months.xlsx

Hi Chole,

Please see the rostered shifts for Cory for the past 8 or so months. Could I please get you to check the actual rosters worked to see if the number of shifts he ended up picking up are added and the shifts he worked are correct. This just gives us a guide as to how he has been rostered the past 8 months and also info about whether he routinely picks up additional shifts, in case he wishes to make a claim for additional shifts.

With regards to recent email communication you have had with Cory can I ask if you can send me an email of your recent communication with Cory about contacting yourself about missed shifts, as I could not find a copy of this email.

Kind Regards
Tammy

From: [Chloe Taylor](#)
To: [Cory Shepherd](#)
Cc: [Tammy Reese](#)
Subject: RE: Catch up 130pm - 15.11.23
Date: Wednesday, 15 November 2023 12:53:06 PM
Attachments: [image001.gif](#)
[image002.gif](#)
[image003.gif](#)
[image004.gif](#)
[image005.gif](#)
[image006.png](#)

Hi Cory,

Sorry I haven't responded I have been in interviews all morning. I would just like to address a few things, and it is mainly regarding rostering. But also, would like to remind you of our shift and break times expectations & some procedures within switchboard.

See you soon.

Chloe

From: Cory Shepherd
Sent: Wednesday, 15 November 2023 8:44 AM
To: Chloe Taylor
Cc: Tammy Reese
Subject: Re: Catch up 130pm - 15.11.23

Hi Chloe,

I've just gone through this. Is it about the roster?

I'm not quite sure why I'd require a support person; I'm comfortable attending on my own.

Get [Outlook for iOS](#)

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>
Sent: Tuesday, November 14, 2023 4:39:00 PM
To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>
Cc: Tammy Reese <Tammy.Reese@health.qld.gov.au>
Subject: Catch up 130pm - 15.11.23

Good afternoon Cory,

As per our conversation last week, I would like to invite you for a quick chat with myself and Tammy tomorrow when you are on shift, would 130pm be suitable? You are more than welcome to bring a support person along if you wish.

Kind regards,

Chloe

Chloe Taylor

A/Switchboard Manager | Switchboard Services

Logan Hospital | **Metro South Health**

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t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au





Roster Concerns

WCRS

Workers' Compensation Regulatory Services
worksafe.qld.gov.au



Renee Matheson

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>
Sent: Wednesday, 8 May 2024 9:08 AM
To: Cory Shepherd
Subject: RE: Roster Concerns

Hi Cory,

Thanks for raising these observations and concerns. I am following up with regards to these with HR for further advice and I will get back to you asap with a response.

Kind Regards
Tammy

From: Cory Shepherd
Sent: Wednesday, May 1, 2024 1:18 PM
To: Tammy Reese
Subject: Re: Roster Concerns

Dear Tammy,

I am writing to further discuss some concerns and observations regarding our operations and rostering process.

Firstly, I appreciate your recent update and the clarity it provided regarding consecutive shifts. I realized that I had been mistakenly referencing this section and it did not support my points towards SECTION 4. HOURS OF WORK 10.4.1 in the OPERATIONS MANUAL.

FOR THE TIME BEING ASSOCIATED WITH SUCH HIGHER DUTIES.

SECTION 4: HOURS OF WORK

10.4.1 Hours of Work

(1) Hours of duty and relevant breaks and off-duty periods shall be in accordance with the CASA mandated fatigue management requirements (CAO 48.1) or Fatigue Management System (FRMS), as adopted by QGAir and applicable at the time. These will be specified in the QGAir Operations Manual.

(2) When a pilot is required to fly in an off-duty period, and the pilot is willing and able to undertake such duty this time may be converted to duty time and time off shall subsequently be granted. Such time off must be taken within twelve months of the day on which the time was worked.

SECTION 5: LEAVE

This section includes a toolkit required by management to manage fatigue and implement appropriate protocols and adhere to the workplace health and safety act. I have attached an image for your reference. It is incumbent upon management not only to prevent an increase in fatigue but also to actively reduce it. This aligns with the best practices outlined and, in the studies, I uploaded to Microsoft Teams over six months ago.

Other shift-related factors that should also be evaluated include shift swaps and remote/phone-access on call shifts.

		Low (1 points)	Moderate (2 points)	High (3 points)	Points
1.	Rostered shift length	<10 hours/week	10 – 14 hours/week	>14 hours per week	
2.	Actual hours worked per week (including overtime)	<48 hours	48-55 hours	>55 hours	
3.	Time worked between breaks	<2 hours	<5 hours	≥5 hours	
4.	Night shift	None	<4 days/week	>4 days per week	
5.	Rostered days off	2 days per week	1 day per week	<1 day per week	
6.	Breaks between shifts	12 hours or more	10 hours or more	<10 hours	
7.	On-call/recall shifts	1 – 2 days per week	3 – 4 days per week	>4 day per week	
8.	Shift rotation	Forward/consistent	Forward/inconsistent	No consistency or direction	
					TOTAL RISK SCORE
0 – 8: LOW RISK		9 – 16: MODERATE RISK		> 16 POINTS: HIGH RISK	

Figure 7: Roster risk assessment matrix

For instance, an increase in night shifts correlates with heightened fatigue levels. Similarly, inconsistent backward rotations exacerbate this issue. I forwarded the toolkit to Chloe last week for review and action but have yet to receive feedback and feel she is not interested.

Furthermore, upon reviewing the rosters in light of the Industrial Relations Act, it is clear that a defined process must be adhered to until a mutually agreed-upon rostering system is in place, in accordance with our individual award (Industrial Relations Section 6: Rosters). I have attached the relevant section below for your reference.

I believe it is crucial to address the ongoing issue of roster control and request facilitation. The numerous errors during Chloe's tenure in management have unfortunately contributed to a decline in our department's performance.

6. Rosters

(a) Rosters will be worked over a 9 day fortnight where rostered hours are a maximum of 9 hours per day.

(b) Unless otherwise agreed, rosters will be implemented as follows for the life of this Agreement.

	S	M	T	W	T	F	S	S	M	T	W	T	F	S
Saturday Roster	O	O	X	X	X	X	X	O	O	RDO	X	X	X	X

62

Sunday Roster	X	X	X	X	X	O	O	X	X	X	RDO	O	O
Monday to Friday	O	X	X	X	X	X	O	O	RDO	X	X	X	O

• Where "O" means rostered off and "X" means rostered on.

• Where industrial parties agree, individual teams may, by mutual agreement, perform work in accordance with alternative rosters as provided for in the Award.

Program management agree to provide notification to the Union and all affected staff of proposed changes to rosters (ie. changes between Saturday, Sunday and Monday/Friday rosters). Management will make all attempts to provide such notification at least one month prior to the earliest possible date of effect.

Except as provided in clause 8, if work is required to be performed on an RDO, overtime rates shall apply.

I am confident that by addressing these issues, we can improve our operational efficiency and enhance the well-being of our team.

I appreciate your attention to these matters. I am eager to contribute to the development of a roster that ensures equitable distribution of shift assignments. I propose to undertake this task over a span of three months. During this period, I can provide detailed insights and reflect wage percentages accurately. For instance, an employee with a .7 designation could anticipate receiving 70% of a full-time employee's wage, and so forth.

Best regards,

Cory

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Date: Friday, 26 April 2024 at 1:52 PM

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Subject: Roster Concerns

Good Afternoon Cory,

In follow up to our meeting on the Tuesday 16th April where you raised concerns about an issue you had with roster out with the previous fortnight's roster line for yourself ending with nights then 3 days off and then the new draft roster with you returning to nights again. I discussed that Chloe was aware of the concern you had with regards to the nights and then nights again on the new roster for the fortnight and Chloe was working to fix this error and would get in touch with you about what alternative shifts she could offer. You also raised a few recent issues you had with how you had been rostered by your line manager and you stated that due to section 15.2(c) Shift work arrangements of the *Award* that you wanted to only be rostered in future as per this section:

Hospital and Health Service General Employees (Queensland Health) Award – State 2015

15.2 Shift work arrangements

- (a) Shift work may be introduced to meet operational requirements. Such shift work shall be worked in accordance with a roster mutually agreed between the employer and the union, in consultation with the employees directly affected. Any discussions concerning the introduction of shift work or roster variations shall be conducted in accordance with the provisions of clause 6.2 of this Award.
- (b) A roster setting out the employee's days of duty and starting and finishing times on such days shall either be displayed in a convenient place or made available electronically to employees at least one calendar week in advance of the roster cycle, provided that notification by 1300 on Monday will be sufficient notification for the week commencing the following Monday.
- (c) A shift worker shall not perform more than two consecutive shifts (e.g. day shift/afternoon shift; afternoon shift/night shift).
- (d) Changes within a roster shall be by agreement between the employer and the employee concerned but, failing agreement, 24 hours' notice of any change in the roster must be given by the employer or double time is to be paid for the employee's next shift.

I think your interpretation of this section is that you only wished to be rostered not more than two consecutive shift types in a row, e.g., Day, Day, or afternoon, afternoon, etc., per fortnight. I discussed that my interpretation of this section was different, and I thought it meant no more than 2 shifts in one day for a shift worker but that I may not necessarily be correct in my interpretation thus we would need to seek HR advice on this. I asked Chloe to follow upon what should be the interpretation of this section with HR and as per the email advice received that I believe Chloe has now sent to you regarding this it states "...the meaning of consecutive shifts is in relation to shifts on the same day, they cannot work 2 shifts in one day. It is not in relation to consecutive days." Thus, based on this interpretation I do believe Chloe's rostering is in compliance with this particular section of the award. I would however be happy to discuss this or any other issues further with if you like just to let me know and we can schedule a time next week or when your next available. If you have immediate issues with the offered roster line for yourself, please discuss this with your line manager in the first instance but your welcome to escalate any concerns to myself. As per the HR email your welcome to discuss any award provisions further with HR as to their interpretation.

Kind Regards

Tammy

Tammy Reese

Director HIMs | Health Information Management Services,

Digital Health and Informatics,

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Queensland Health

Fatigue risk management systems

Implementation guideline QH-GDL-401-3.3:2021



Queensland
Government

Contents

1	Introduction	2
1.1	About this guideline	2
	Additional sources of information and assistance	3
2	Defining fatigue	4
	What is fatigue?	4
	Causes and consequences	4
	Sleep and fatigue – meeting our basic physiological needs	5
3	Fatigue risk management elements	7
3.1	Overview	7
	FRMS system elements	7
4	Fatigue risk management system implementation	9
4.1	Governance	9
4.2	Planning for fatigue risk management	10
4.3	Fatigue risk management	12
	Defences in depth	12
	The risk assessment process	14
	Risk control strategies	18
5	Incident management	22
5.1	Hazard and incident reporting	22
5.2	Incident response and investigation	23
6	Monitoring and assurance	23
6.1	Performance indicators	24
6.2	Continuous improvement	25
7	Safety promotion	25
7.1	Legislative obligations for consultation and training	26
7.2	Planning for communication of the FRMS	26
8	Legislation	27
9	Supporting documents	27
10	Definitions	28
11	Version control	30
12	References	30

1 Introduction

1.1 About this guideline

This guideline has been developed to support all Queensland Health workplaces to identify and manage fatigue risks. It draws on lessons learnt from over a decade of implementing fatigue risk management systems (FRMSs) in Queensland Hospital and Health Services (HHSs) and from proven approaches to safety risk management.

In Queensland, the *Work Health and Safety Act 2011* (WHS Act) is the instrument that regulates safety in workplaces. It imposes specific duties on a 'person conducting a business or undertaking' (PCBU) including a duty to ensure 'so far as reasonably practicable' that workers are not put at risk from work carried out by the business.

The WHS Act also imposes specific duties on a worker whilst at work. Workers must take 'reasonable care' for their own and others health and safety and comply so far as 'reasonably able' with any reasonable instruction given by the PCBU.

This guideline is designed to provide information to all levels of the organisation to empower everyone to champion fatigue management. It outlines procedures and processes to be developed by each accountability area to mitigate fatigue risk and to operationalise the Queensland Health Health safety and wellbeing management system (SMS).

Historical context

In January 2002, 10 year old Elise Neville attended a Queensland Health hospital emergency department with a head injury after having fallen from a bunkbed. She was examined by a doctor working the twentieth hour of a 24-hour shift and was subsequently sent home. Although it was not immediately identified, Elise had suffered an extradural haematoma and timely and appropriate treatment was not provided. Elise died some days later.

An investigation by the Queensland Ombudsman (2006) found that one of the contributing factors to Elise's death was fatigue caused by long hours of work. As a result, the Ombudsman recommended that actions be taken by Queensland Health to manage fatigue risks in its medical workforce.

In response Queensland Health embarked on a major project to address fatigue in the medical workforce. The "Alert Doctors Strategy" project undertook case studies of a number of individual departments and whole facilities leading to the implementation of fatigue risk management strategies suitable to each unique work environment. The case studies provided great organisational insights that informed the development of a resource pack and policy for managing fatigue in the medical workforce. The policy now applies to all employees and complements the overarching Queensland Health *Health safety and wellbeing management system*.

Principles for managing fatigue

- A shared approach between the organisation and workers
- A sound risk management approach through the application of the Defences in Depth model for fatigue risk management
- A systemic approach that is incorporated into core business operations
- An aware and informed workforce approach
- An integrated approach that achieves consistency with existing health, safety and wellbeing management systems

Adoption of the advice in this guideline will help to manage fatigue-related risk, improve worker health and well-being and reduce fatigue hazards and fatigue-related workplace incidents. It will also support regulatory compliance.

This guideline provides a basic framework and the core elements to assist in the design, implementation and ongoing evaluation of an FRMS. It provides guidance on the key elements of the system and suggests risk control measures.

The guideline applies the principles outlined in the Fatigue risk management policy – I1 (QH-POL-171) and supports integration with other safety systems, principally the Queensland Health *Health, safety and wellbeing management system* (SMS).

This guideline is designed to be used throughout Queensland Health to support implementation of local fatigue risk management systems.

Throughout this resource specific terms are used to describe aspects of fatigue and fatigue risk management systems. These terms are defined in section 10 of the guideline.

Additional sources of information and assistance

While this guideline provides the foundations to build and operate an FRMS, a range of other resources should be drawn upon for the day-to-day operation of an FRMS.

Scientific research continues to uncover new information not only about the impacts of fatigue, but also about control measures, work design factors and safety management and wellbeing in general.

In recent years, individual discipline-based journals have acknowledged fatigue as a factor that influences health, safety and wellbeing, workplace productivity and the delivery of quality health care.

Within Queensland Health a range of additional and interrelated resources and support is available. A comprehensive list is provided in section 9 of this guideline.

2 Defining fatigue

What is fatigue?

Fatigue is a state of impaired physical and/or mental performance and lowered alertness arising as a result or combination of physical and mental work, health and psychosocial factors or inadequate restorative sleep. (Schutte 2009)

It is a decreased capacity for physical and/or mental activity resulting from imbalances of the resources required to perform the activity (Aaronson et al. 1999).

Fatigue can result from shift work, travel, driving to remote locations, crises and disaster events as well as non-work-related situations. It does not discriminate and can impact everyone from senior executives through to junior clinical and non-clinical staff. In almost each situation fatigue results from long work hours and lack of quality sleep.

A body of scientific evidence has clearly demonstrated the relationship between fatigue and performance degradation including:

- a decrease in cognitive function and task performance
- increases in error and accident rates
- a reduction in safety.

Fatigue can lead to wide-ranging forms of impaired performance and can have negative impacts on both the delivery of health care and the health, safety and well-being of individuals.

Causes and consequences

Fatigue is a common and unavoidable by-product of 24-hour delivery of patient care. Working in shifts is heavily relied upon to deliver health services. Consequently, fatigue tends to impact shift-workers and is often perceived to be attributed only to shift work. This perception is founded because humans are diurnal in nature, meaning that they have evolved to be awake during the day and sleep during the night. This diurnal sleep-wake pattern is entrenched in our circadian rhythm, our 24-hour body clock. The very nature of shift work disrupts the circadian rhythm and effects a range of behaviours and conditions which in turn affect a person's physiological and psychological state. If not managed, this disruption can lead to accidents, injuries and chronic disease. The potential health impacts of shift work are depicted in Figure 1.

It is however important to note that many other work-related and personal factors can also contribute to fatigue (Figure 2). These include:

- workplace stressors such as demanding or high-pressure roles or tasks
- inadequate breaks between shifts
- medical conditions or illness
- non-work factors such as family commitments, lifestyle, diet, financial or relationship difficulties and some medications.

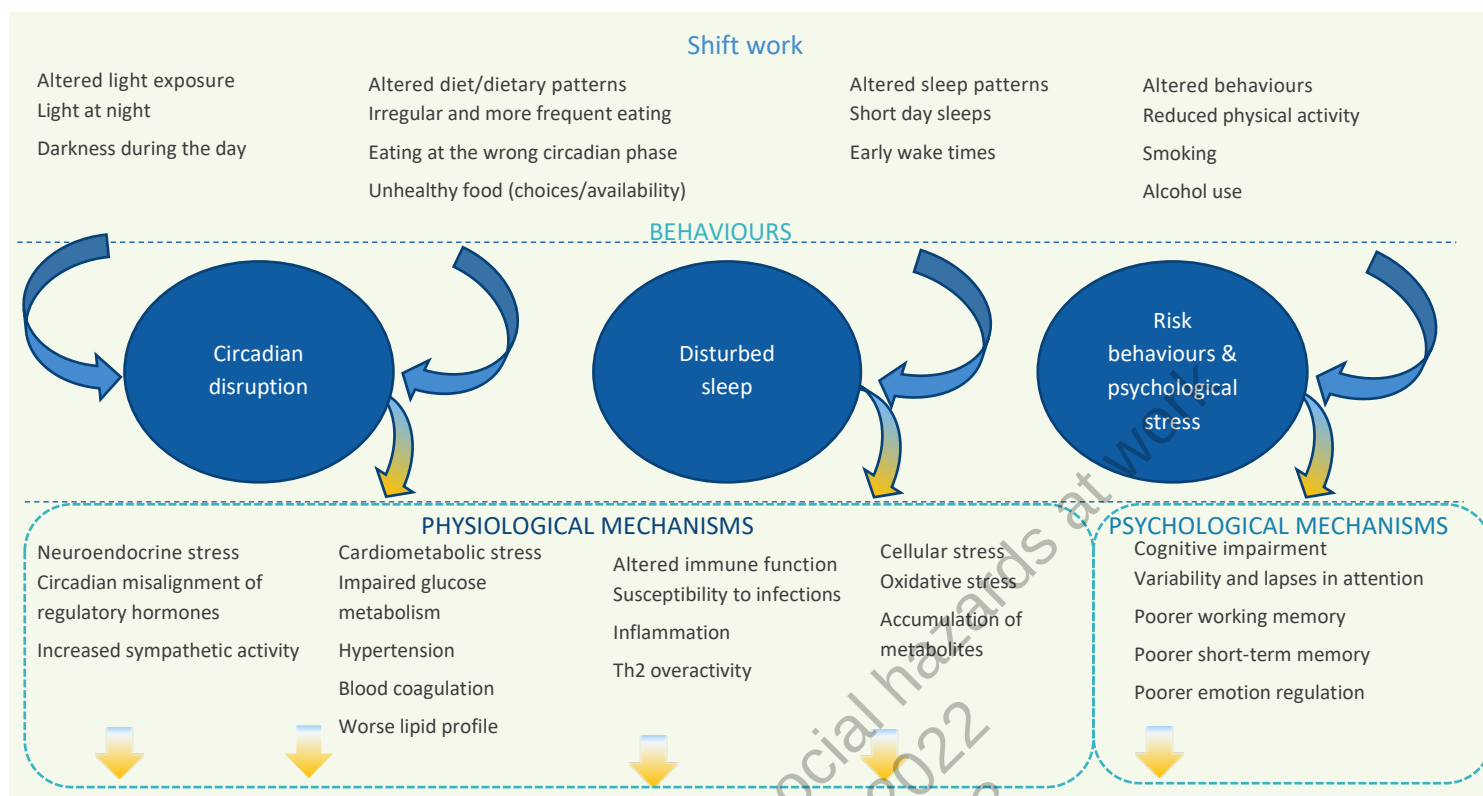


Figure 1: Pathways by which shift-work may increase illness and injury risk (adapted from Kecklund & Axelsson 2016)

Sleep and fatigue – meeting our basic physiological needs

Among our most basic needs is the need for rest. More specifically, sleep is a basic physiological need, alongside hydration, nutrition and other essential processes that support life. To highlight the detrimental effects of insufficient sleep, recent research indicates sleep is necessary to remove waste products from the brain and that a lack of sleep may be a risk factor for the development of Alzheimer's disease (Komaroff 2021).

Sleep is fundamental to health and wellbeing and disruption to our normal sleep requirements has both short term and long-term impacts.

All human adults require approximately seven to nine hours of sleep per night. Research has demonstrated that sleep disturbances whether due to work-related factors such as shift work or personal factors such as sleep apnoea, can result in impaired performance and loss of productivity (Rosekind et al. 2010).

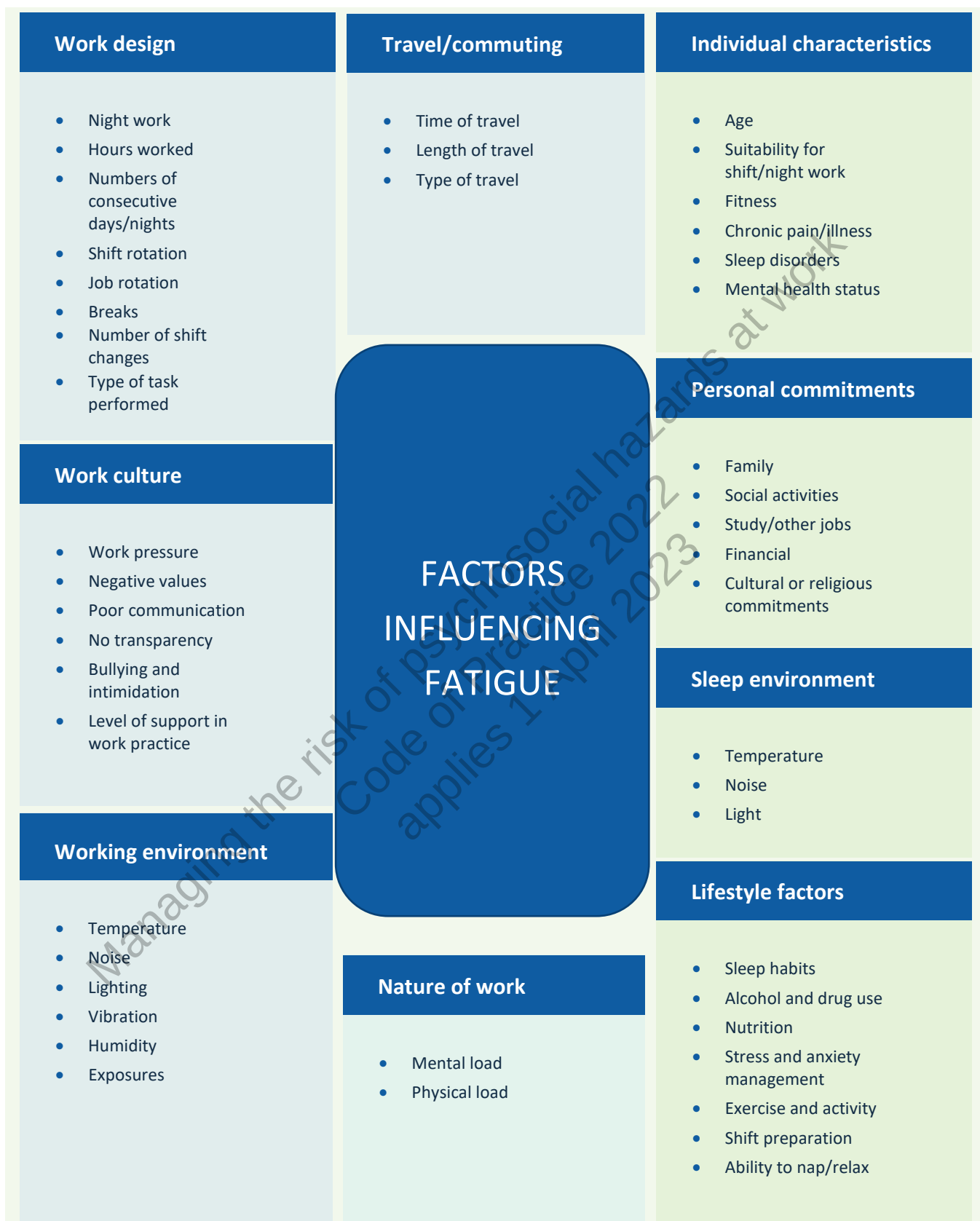


Figure 2: Fatigue risk factors

3 Fatigue risk management elements

3.1 Overview

The FRMS is a framework and set of formal processes designed to systematically identify areas where fatigue poses a risk and effectively the risk through appropriate control measures. The FRMS tailors the principles of a systems-thinking approach typically applied in health safety and wellbeing management systems to fatigue risk management. Central to successful implementation of a risk management system is:

- planning
- commitment
- consultation
- clarity of roles and responsibilities
- documentation
- monitoring and evaluation
- review

Queensland Health's Health safety and wellbeing management system (SMS) consists of a policy and seven associated standards which give regard to these matters. Implementation of the standards is supported by guidelines and other resources. The standards outline the minimum requirements for each of the SMS elements.

Accountability areas across Queensland Health are responsible for developing local documentation, procedures and processes to ensure the requirements of the SMS are met.

FRMS system elements

The FRMS consists of six key elements that collectively are a means by which to monitor and manage fatigue-related risk based on data, operational knowledge and scientific evidence.

- **Governance structure** - Good governance requires commitment from a range of persons including the executive, directors, line managers and supervisors and the individual members of the workforce. The establishment of a local working group or similar (e.g. the accountability area's HSW committee) to oversee fatigue-related risk management is an important aspect of the governance structure.
- **Planning** – Planning for fatigue risk management takes into account criteria such as the context of the organisation, relevant stakeholders and how data and information can be gathered to identify areas of risk and assess the level of risk. Planning processes should consider also how best to integrate actions into existing business and processes.
- **Fatigue risk management** - Understanding where fatigue-related risks are present is central to the FRMS. The risk assessment process identifies where fatigue-related risk is highest by considering a range of factors including hours of work, the type of work and the number of people likely to be affected. The fatigue risk assessment also considers existing measures used to manage fatigue-related risks and whether they are adequate. Risk mitigation should be guided by the **Defences in**

depth model which is discussed in greater detail in section 4 of this guideline. The policies, practices and procedures both at a local level and system wide also are control measures at each level of the framework.

- **Incident management** – Reporting of and responding to incidents is important for understanding where risk may be emerging or to prompt risk reassessment or evaluation of existing control measures. It is also necessary to be able to investigate the cause and effect of the incident and implement additional or alternative risk mitigation measures.
- **Monitoring and assurance** - Monitoring the effectiveness of controls and the outcomes of fatigue risk management are important aspects of the FRMS and should be incorporated into each accountability area's safety assurance framework. Monitoring is usually data-driven and relies on accurate reporting systems and well-designed performance indicators (PIs). Monitoring also presents an opportunity for review and evaluation of controls and to consult with the workforce on the effectiveness of strategies implemented. This also assures workers that the PCBU regards that their safety and wellbeing is important. It also demonstrates that the organisation is meeting its health, safety and wellbeing obligations.
- **Safety promotion** – Consultation is essential for obtaining information about potential issues and suggestions for suitable risk treatments. Training, education and instruction are all important for communicating information about fatigue-related risks to workers and to keep them informed about management strategies and individual and organisational responsibilities. Training and education are also important control measures.

SMS framework		FRMS framework
Health, safety and wellbeing policy	↔	Fatigue risk management policy
Governance standard	↔	Governance structure
Planning standard	↔	Planning for fatigue risk management
Risk management standard	↔	Fatigue risk management
Incident response standard	↔	Incident management
Monitoring, evaluation and performance review standard	↔	Monitoring and assurance
Consultation standard	↔	Safety promotion and planning for fatigue risk management

Table 1: Comparison of the SMS and FRMS frameworks

Key elements of the FRMS align with the SMS (Table 1) and development of local processes and associated documentation should give regard to the overarching SMS in place. Implementation of the FRMS framework supports compliance with the SMS framework.

4 Fatigue risk management system implementation

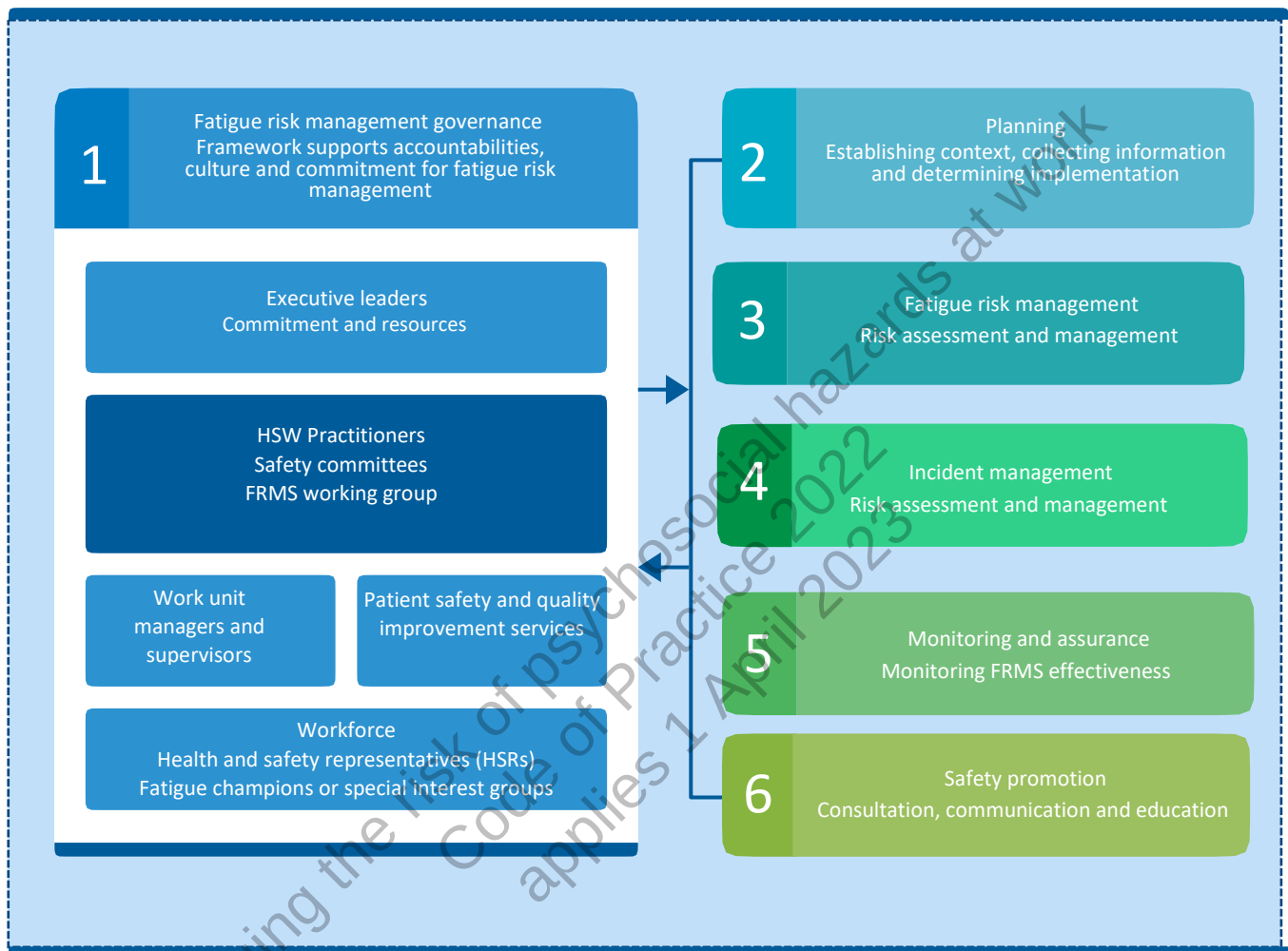


Figure 3: The FRMS framework

4.1 Governance

Accountability areas are responsible for ensuring that fatigue risks are effectively managed so as not to compromise the safety and wellbeing of workers, patients and others. The Fatigue risk management policy – I1 (QH-POL-171) sets out roles and responsibilities and auditable criteria for compliance with the policy.

Organisational level roles and responsibilities are outlined in the Health, safety and wellbeing governance standard QH-IMP-401-6:2021. The standard also sets out the governance framework for health, safety and wellbeing in the workplace. It provides for WHS committees and focus groups and associated reporting structures.

Any **local fatigue working groups** established to oversee the implementation of the FRMS should be established within the existing HSW governance framework for the accountability area.

Historically **local fatigue champions** have been encouraged to promote fatigue management policies, procedures and processes and bring about cultural change within work units. Local fatigue champions can be individuals, health and safety representatives (HSRs) or groups within particular occupational streams that actively promote health, safety and wellbeing for their profession or the organisation. Examples of such groups are the Healthcare Improvement Community of Practice (HICOP) group and the Queensland Emergency Department Strategic Advisory Panel (QEDSAP) Wellbeing Working Group. Inclusion of such individuals or groups to support established governance structures, communication and consultation is strongly recommended.

Opportunities for integration of governance arrangements with local **patient safety and quality improvement service** should also be explored. This service is responsible for investigation of clinical incidents including those where worker fatigue may be a contributing factor. They also oversee accreditation requirements relating to the National Safety and Quality Health Service Standards which may support some FRMS requirements.

Figure 3 provides an example of the various entities that may be involved in FRMS governance, however some aspects will vary depending on systems and groups that exist within the accountability area.

4.2 Planning for fatigue risk management

When planning for implementation of the FRMS a consultation process should be established with workers who are most likely to be at risk of fatigue based on work type, work design and work environmental factors. Consultation helps to identify issues and solutions that are most likely to be adopted by the persons impacted. It also helps to convey the concept of shared responsibility. Beyond this, consultation is required under the WHS Act and as such, must comply with legislative requirements and the Health, safety and wellbeing consultation standard (QH-IMP-401-2:2020).

Other factors that should be considered when planning for fatigue risk management are outlined in Table 2.

Factors	Key considerations
Work, health and safety legislation	No prescribed requirements specific to fatigue in the Act or Regulation. A person conducting a business or undertaking (PCBU) must, among other things, provide safe systems of work, identify hazards and manage risks in accordance with the prescribed hierarchy of controls. This includes managing risks to both physical and psychological health. The provisions of relevant codes of practice must be adhered to.
Fatigue risk management policy – 11 (QH-POL-171)	Sets out principles for managing fatigue and includes responsibilities of all staff. It is fundamental to the requirement for commitment from executive leaders and the concept of shared responsibility for fatigue management. Among the responsibilities for executive leaders outlined in the policy are requirements to: • provide leadership and stewardship for fatigue risk management • encourage a positive safety culture

	• support the implementation of the FRMS • own fatigue risks and advise of barriers preventing risk reduction.
Health, safety and wellbeing plans	Under the Health, safety and wellbeing planning standard (QH-IMP-401-1:2020) there is a requirement to establish plans which detail the objectives and strategies for health, safety and wellbeing. The plans should be developed in consultation with workers and shared duty holders and communicated accordingly.
Understanding our business	This enables a targeted approach to risk assessment and risk management. An environmental scan of work units and occupations informs planning and decision making in fatigue risk management. Consider: • staffing arrangements/resourcing • worker demographics • occupational streams • geographical location • the nature of the service being delivered • political and public expectations • the physical work environment • irregularities in operations (on-call, shifts, overtime, emergency events) • the nature of tasks being performed • experience and capability of workers • data collection (e.g. incident and payroll reports).
Understanding the scientific principles of fatigue management	• The need for sleep • Sleep debt and recovery • Circadian effects on sleep and performance • The influence of shift work • The influence of workload • Impact of environmental conditions and tasks • Impact of stress and other psychological stressors.
Industrial instruments	Awards and enterprise agreements specify hours of work and other requirements that support fatigue management.
Fatigue risk management processes	• Hazard identification or situations or conditions that create or contribute to fatigue risk • Risk assessment • Risk mitigation through implementation of risk controls • Risk register to record findings, actions and outcomes.
Monitoring and assurance	Determine what is in place or available to assess the extent and effectiveness of FRMS implementation and risk mitigation strategies.

Safety promotion	Training, information and instruction to inform all parties of the FRMS, responsibilities, impacts of fatigue, management strategies and reporting requirements. The aim of safety promotion is to encourage a culture where all personnel apply the actions and behaviours that support fatigue risk reduction.
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Table 2: Factors to consider when planning for fatigue risk management

4.3 Fatigue risk management

Fatigue risk management consists of:

- hazard identification
- risk assessment
- implementation of risk mitigation strategies
- review of effectiveness of control measures.

“If there is a fatigue related accident there is probably a series of causally related events that led up to that incident.” (Dawson 2019)

In planning and developing the FRMS, an environmental scan of the business should already have been undertaken and areas where fatigue poses a risk identified. If this has not occurred, worker consultation and data analysis should take place to identify work areas in the accountability area where fatigue may be a risk.

Defences in depth

Queensland Health has adopted the **Defences in depth** model for fatigue hazard identification and risk mitigation (Figure 4). This model has been adopted because it guides the assessment of fatigue risk by looking at five circumstances that follow a trajectory that leads to a fatigue-related incident. At each level, related fatigue risk factors (from Figure 2) should be examined and corresponding control measures applied.

Each level falls into one of three types of processes:

- Predictive
- Proactive
- Reactive.

Implementing multiple interventions helps to limit the likelihood of fatigue.

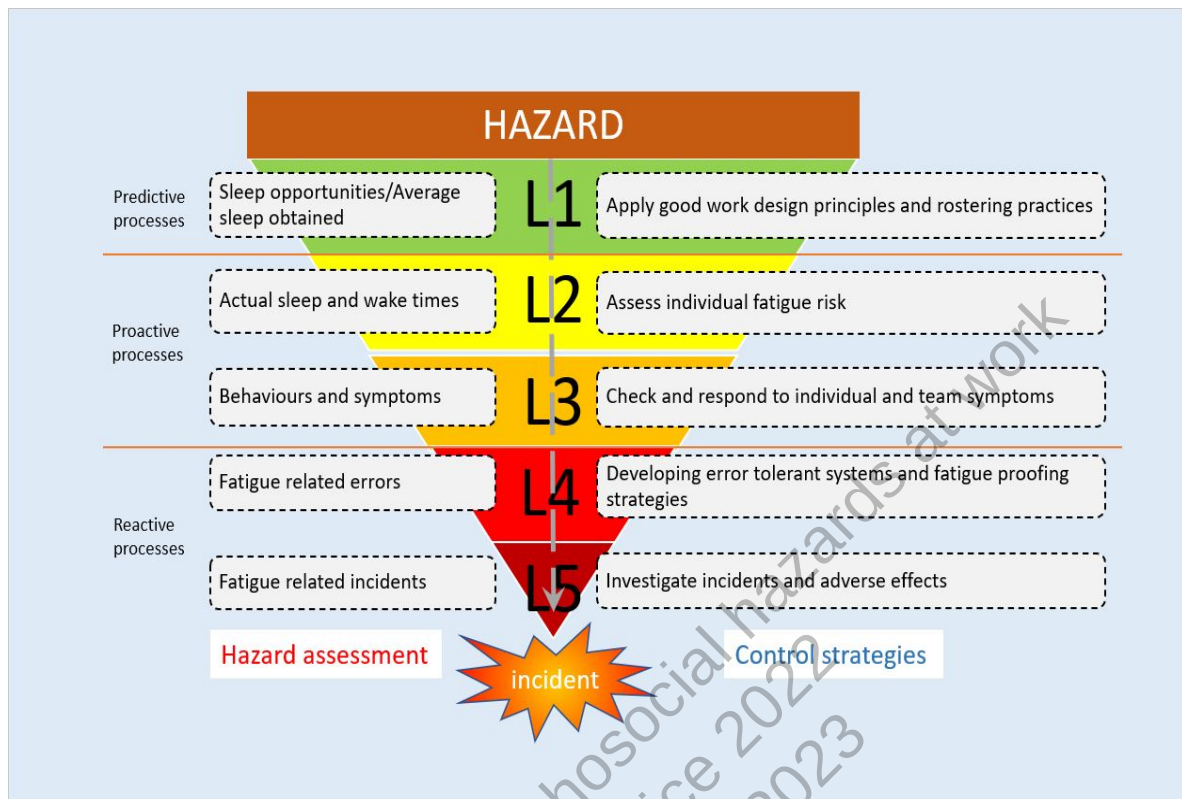


Figure 4: Defences in depth fatigue risk management model

Relationship to the hierarchy of controls

The *Work Health and Safety Regulation 2011* and *How to manage work health and safety risks Code of Practice 2021* require that risks to health and safety be eliminated to the extent that it is reasonably practicable to do so. Where elimination is not possible, other methods should be applied in order of effectiveness from the highest level of protection and reliability to the lowest, or in combination to minimise risk so far as reasonably practicable. The principles of the Hierarchy of control are similar to the Defence in depth model in that elimination of the hazard or risk is preferred but where this is not achievable, there is a range of control options that have a descending order of effectiveness. A comparison of the two models is depicted at Figure 5.

Understanding the differences in models and being able to explain the application of the Defences in depth model is important in the event that there is a need to demonstrate to a WHS regulator how fatigue risks are managed.

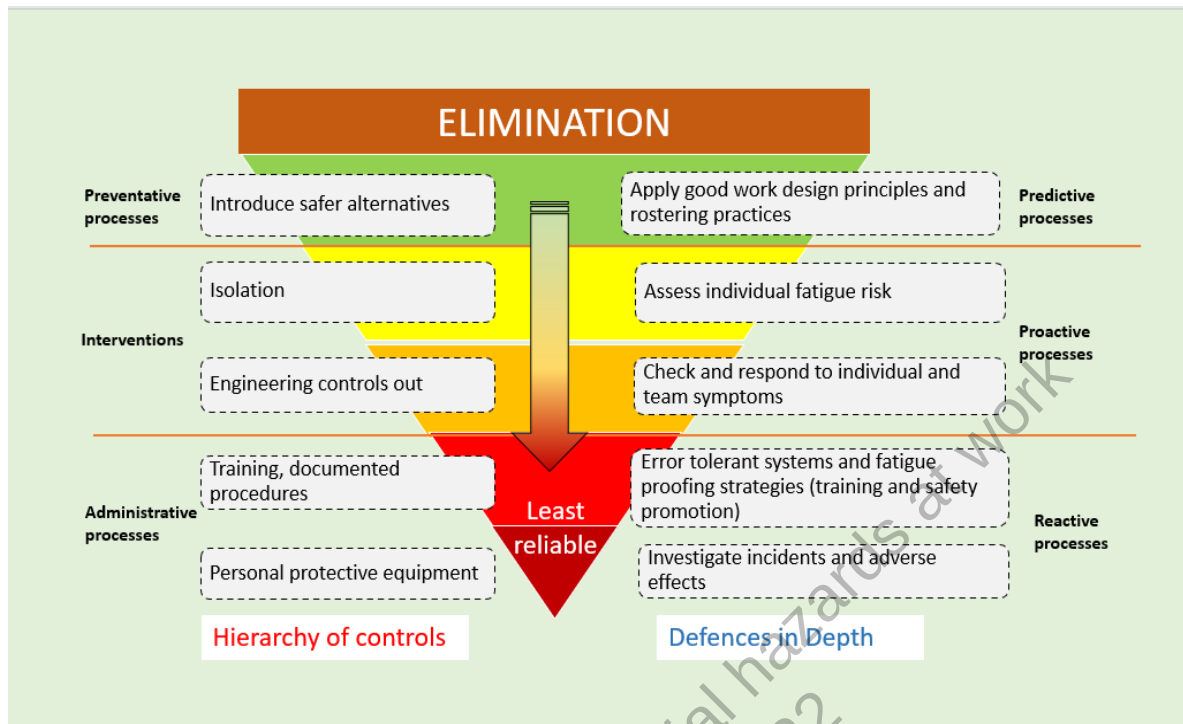


Figure 5: A comparison of the traditional hierarchy of controls and Defences in depth models of risk controls

The risk assessment process

Hazard identification

Fatigue is a hazard because it can cause harm to an individual. However, it is critical to recognise the factors that contribute to fatigue because this is the level at which measures to prevent fatigue can be implemented. Factors to be considered in fatigue hazard assessment are described in the Defences in depth model and are:

- Whether there are opportunities for adequate sleep
- Actual sleep and wake times versus rostered work time
- Identification of behaviours and symptoms that indicate an individual may be at risk of fatigue
- Whether fatigue has contributed to errors
- Whether there are fatigue related incidents or near misses

Risk assessment

Fatigue can impact our workers and our consumers in a variety of settings. Risk can be assessed by work unit, by occupational stream, at an individual level and through the identification of new or emerging hazards or risk factors introduced through changes to work design or the work environment. In all cases the tasks being undertaken by workers must also be taken into consideration.

Determining the level of risk is typically done using a risk matrix which assigns a risk rating based on the likelihood that an event will happen and the severity or consequences of the event. The impact of Fatigue may have multiple consequences. Potential consequences to consider when determining the level of risk posed by fatigue include:

- psychological harm (stress, burnout)
- patient harm resulting from errors caused by fatigue
- musculoskeletal and other physiological injuries
- damage to property such as vehicles or equipment.

Assessing fatigue risk can be challenging because as noted in Figure 2, there are numerous factors that contribute to fatigue, some of which are personal and not always obvious when workplace risk assessments are being carried out. Hence assessments should be carried out at both a work unit level and an individual level.

When undertaking the risk assessment, information about contributing factors and control measures currently in place should be gathered. Types of data and information that can inform the risk assessment are shown at Figure 6.

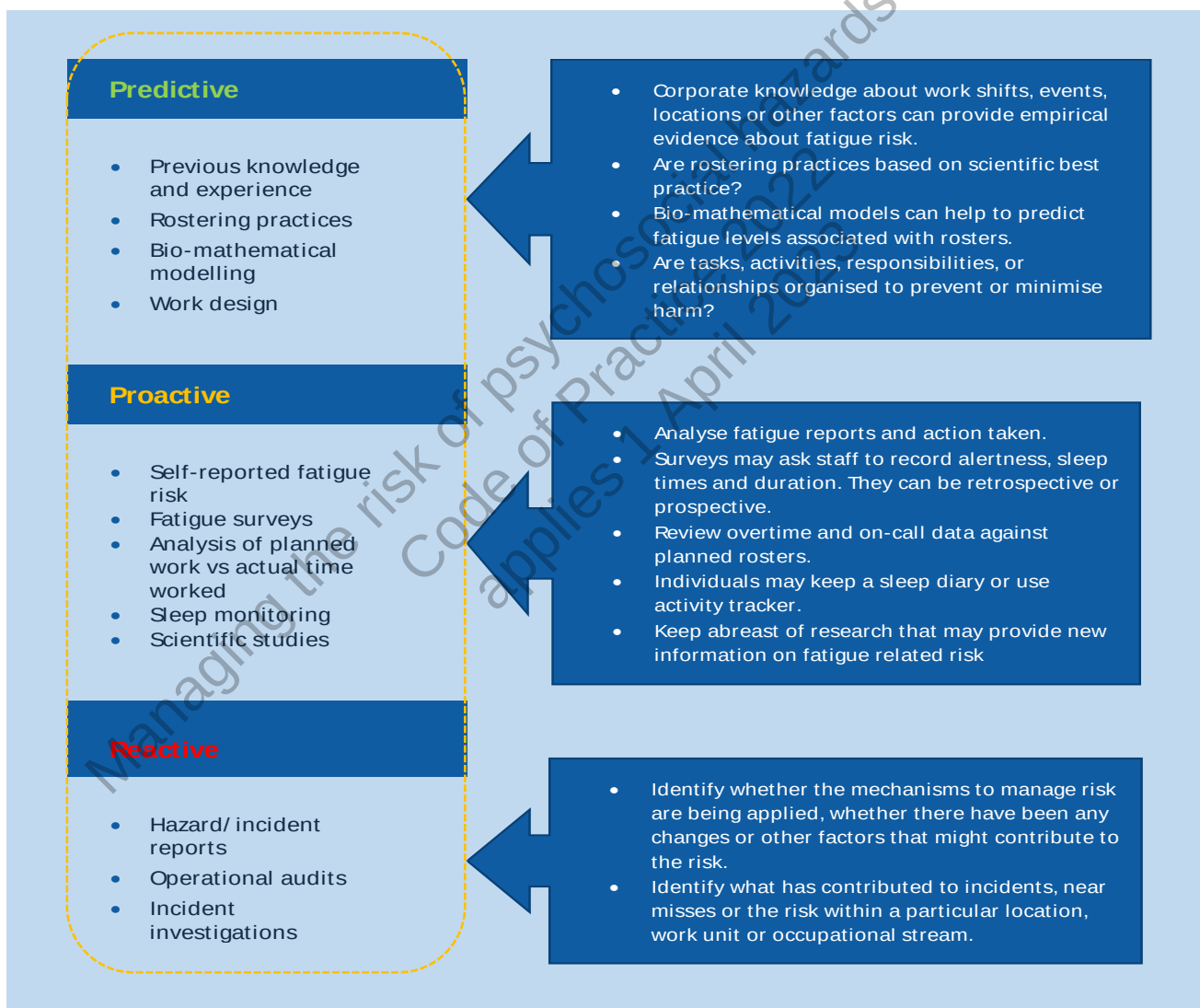


Figure 6: Data and information for fatigue risk-factors

Level 1 Predictive processes: work unit assessments

During the planning and hazard identification phases fatigue may have been identified as a likely workplace hazard for particular work units. Where this is the case, the risk assessment should focus on the work unit as a whole. This risk assessment should consider the information gathered during the planning and hazard identification phases.

Good work design and rostering is a key Level 1 control in the Defences in depth hierarchy and roster appraisal is an important aspect of the work unit assessment. Figure 7 shows a roster assessment tool. Rostering should apply evidence-based rostering practices proven to mitigate fatigue risks as well as any requirements specified in awards and industrial agreements and recommendations set out in relevant codes of practice.

Key rostering elements that should be considered are:

- rostered hours
- actual hours worked
- shift duration
- length of breaks between shifts
- rostered days off
- the number of night shifts
- on-call/recall shifts
- shift rotation.

Other shift-related factors that should also be evaluated include shift swaps and remote/phone-access on call shifts.

		Low (1 points)	Moderate (2 points)	High (3 points)	Points
1.	Rostered shift length	<10 hours/week	10 – 14 hours/week	>14 hours per week	
2.	Actual hours worked per week (including overtime)	<48 hours	48-55 hours	>55 hours	
3.	Time worked between breaks	<2 hours	<5 hours	≥5 hours	
4.	Night shift	None	<4 days/week	>4 days per week	
5.	Rostered days off	2 days per week	1 day per week	<1 day per week	
6.	Breaks between shifts	12 hours or more	10 hours or more	<10 hours	
7.	On-call/recall shifts	1 – 2 days per week	3 – 4 days per week	>4 day per week	
8.	Shift rotation	Forward/consistent	Forward/inconsistent	No consistency or direction	
TOTAL RISK SCORE					
0 – 8: LOW RISK 9 – 16: MODERATE RISK > 16 POINTS: HIGH RISK					

Figure 7: Roster risk assessment matrix

Biomathematical modelling tools may also be helpful for predicting or estimating fatigue likelihood based on sleep-wake and work-rest data. Application of these may be useful in determining whether the fatigue risk levels of rostered work hours are tolerable.

Consideration should also be given to staffing levels, the types of occupational streams, tasks undertaken and environmental factors. Factors such as mental or physical intensity of a task, repetitive work and poor ventilation or lighting can all contribute to elevated fatigue risk.

Collectively this information can be used to assess the overall fatigue risk for the work unit. The risk matrix used to assess other health and safety risks can be applied to determine the fatigue risk rating for the work unit.

Levels 2 and 3 proactive processes: assessment of individuals

Assessment of proactive processes typically are aimed at fatigue risk that may be expected to occur during work operations. They are a more dynamic type of assessment which introduce the concept of shared responsibility and in part rely on individual workers to actively monitor, assess and self-report their own fatigue levels and fitness for work at commencement of and during their work shift.

The effectiveness of Level 2 and 3 processes is impacted by numerous factors including the level of workplace support, organisational priorities, organisational consequences for self-reporting fatigue, and the individual's willingness to participate and their personal circumstances.

Various tools can be used for individual fatigue risk assessment. These include:

- keeping a sleep wake diary
- completing a fatigue survey
- using a monitoring device such as a smart watch with a sleep monitoring feature
- using a fatigue app
- doing a self-assessment using an individual fatigue likelihood calculator tool.

Additionally, work unit supervisors and colleagues should also monitor other workers' behaviours and symptoms. One tool that can be used for this is the Samn-Perelli fatigue checklist (Figure 8).

The Samn-Perelli fatigue checklist is a subjective assessment that aims to identify fatigue symptoms that may occur despite appropriate rostering and sleep.

Relevant tools and templates are listed in Section 9 of this guideline and available on QHEPS.

LOW RISK	• Fully alert • Very lively
MODERATE RISK	• Feeling okay • A little tired
HIGH RISK	• Moderately tired • Extremely tired
EXTREME RISK	• Completely exhausted

Figure 8: Samn-Perelli fatigue checklist

Levels 4 and 5 reactive processes: assessment of incidents

This assessment occurs during or after an incident or event and is important for:

- determining whether existing risk management strategies are effective
- development of error tolerant systems of work
- informing work unit risk assessments
- monitoring and assurance requirements
- system reform.

An analysis of errors and incidents should consider whether fatigue was a causal or contributing factor. The following Queensland Health methodologies may be applied for incident analysis:

- The Patient Safety clinical incident analysis process
- The HSW incident investigation process.

Risk control strategies

Where a risk assessment has determined that the level of risk is unacceptable, appropriate control strategies need to be implemented. Often more than one strategy may be implemented to manage the identified risk. The Defences in depth model also supports two approaches for reducing fatigue related risk. These are:

- Fatigue reduction strategies which correspond to the controls that are applied from Levels 1 to 3 of the model.
- Fatigue proofing strategies which correspond to Level 4 controls.

A range of potential strategies are outlined in this section. However, they may not always be appropriate due to a range of factors including resourcing constraints, geographical location or acute and protracted emergency or disaster events. Staff should be consulted to tailor risk mitigation solutions that support both organisational and worker needs.

Defences in depth: Level 1 – predictive controls

Level 1 controls must aim at providing adequate sleep opportunity. The key control strategy is having a roster design that meets best practice rostering guidelines and any mandated requirements of industrial instruments. Dynamic variables that need to be considered include:

- **Worker availability** – workers may be unavailable due to illness, training requirements, leave or personal matters. Consider how often this happens and how long for and whether it is possible to fill the position without imposing additional hours on the existing staff. Accessing additional workers from a casual employee pool or agency staff may sometimes be required to appropriately fill a roster without imposing additional hours on permanent rostered staff.
- **Shift swaps** – informal shift swapping can inadvertently increase fatigue-related risk. A formal process to manage and approve shift swaps should be developed, implemented and monitored.
- **On-call** – Industrial instruments stipulate some requirements for the management of on-call and recall, however on-call shifts can present a unique set of challenges. Studies suggest that the most

significant impact of being on-call is the stress that is brought on by the anticipation of being called. Being on-call also places restrictions on what the individual is able to do during what would normally be a rest period. This can result in another layer of stress that contributes to the fatigue risk. The time of day that the on-call is scheduled for should also be considered. There is evidence that suggests that night-time on-call can create a greater fatigue risk than daytime on-call (Ziebertz et al. 2015). To manage this risk factor, consideration should be given to limiting the frequency of on-call shifts as well as the time of day that the shifts are scheduled for.

Bio-mathematical modelling can also be used to predict fatigue-related risk. Often this is based on prior work schedules.

Defences in depth: Level 2 and 3 – proactive controls

Proactive controls apply predominantly to individuals. Successful implementation of proactive controls at an organisational level relies strongly on promoting a safety culture where individuals feel confident that they can report their fitness for work without fear of reprisal.

The fatigue risk management system should include tools to support an individual's assessment of their fitness for work. Formal procedures must be implemented to support those individuals in situations where the risks associated with fatigue outweigh the risks associated with continuing to work.

When an individual has assessed their fatigue risk, control measures can be applied accordingly. Examples are provided in Table 3.

Prior wake time	Risk level	Controls
<12 hours	Low	No specific controls necessary. A 'business as usual' approach should take place. Normal monitoring should continue. Strategies include: • individual sleep wake assessment • monitoring for symptoms • monitoring for performance degradation.
12 – 14 hours	Moderate	Initiate moderate fatigue mitigation actions A moderate level of fatigue risk indicates a potential for fatigue to occur. Frequency of monitoring should be increased and fatigue countermeasures implemented. Monitoring strategies As for low risk Countermeasures (Individual controls) • napping • rest breaks • adequate hydration and appropriate nutrition • task rotation • perform light physical activity • take a break from repetitive tasks or screen intensive work.

Prior wake time	Risk level	Controls
14 – 16 hours	High	Initiate moderate fatigue mitigation actions A high level of fatigue risk indicates that fatigue is highly likely to occur. Risk mitigation strategies are critical to reduce the risk of harm. Monitoring strategies As for low risk Countermeasures (Individual controls) As for moderate risk Team-based controls • declaration of fatigue-risk to the team • task reallocation • increased team cross-checking • additional consultation on critical decisions or actions. This may include priority access to on-call facilities. • allocating another individual to take the lead on key tasks • deferring non-urgent tasks • increase supervision • use alternative systems that may be in place to deliver the necessary service where possible (e.g. Telehealth) • reallocate or reschedule tasks where possible • priority access to napping arrangements • access to safe commuting arrangements.
>16 hours	Extreme	Risk is unacceptable. No individual to work beyond this threshold. An extreme level of fatigue risk indicates that the risks associated with fatigue are critical and the potential for harm is such that work should not continue. Concessions should only be considered in exceptional circumstances and must be escalated to an executive leader(s) for approval. An escalation and approval process should be established clearly outlining requirements. Monitoring strategies As for moderate risk Fatigue risk management decision process • work must not continue unless there are exceptional circumstances where to discontinue work would result in more dire consequences. • decision to be made in consultation with the worker(s) and executive leaders. All viable alternatives must be explored. • decision must be documented as part of the incident reporting process. Specific fatigue controls • individual counter-measures as per moderate risk